National Maternal Health Analysis

Barriers to Institutional Delivery

Data-driven root cause analysis revealing hidden bottlenecks preventing mothers from accessing Institutional care

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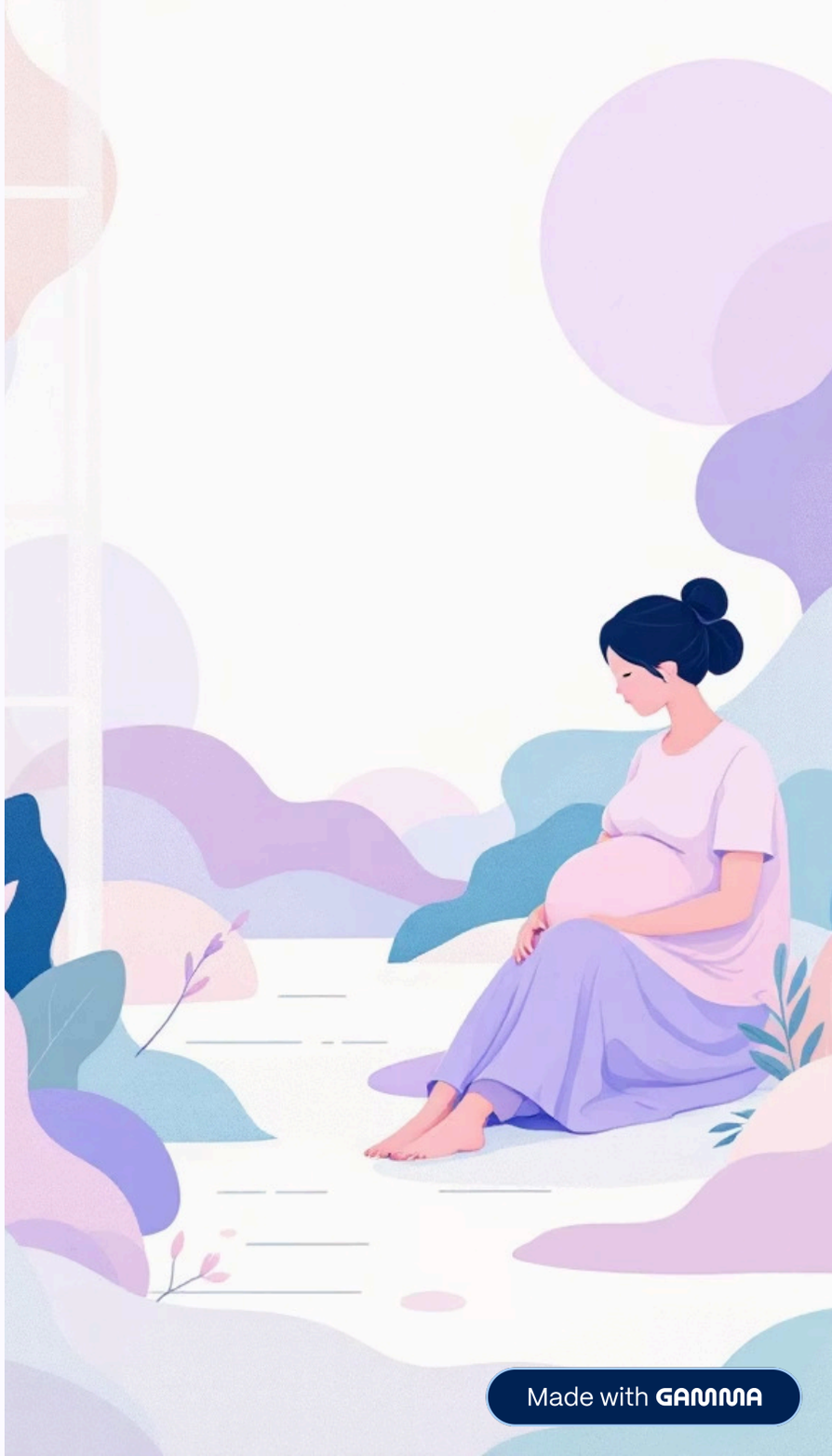
Problem Statement

Despite widespread national initiatives aimed at enhancing maternal healthcare access, a significant challenge persists: approximately 20% of deliveries in designated critical districts still occur outside of medical facilities. This persistent gap highlights that the mere presence of healthcare infrastructure does not guarantee its utilization, leading to suboptimal health outcomes for mothers and newborns.

Our Objective

Our primary goal is to conduct a comprehensive root cause analysis to identify the "hidden bottlenecks" that deter expectant mothers from accessing and consistently utilizing institutional care for childbirth. This involves delving deeper than surface-level observations to uncover systemic and operational barriers.

A key insight guiding this project is that true access to maternal healthcare extends beyond building new facilities. It crucially involves ensuring seamless service continuity throughout the entire pregnancy and postpartum period, coupled with robust financial protection mechanisms to alleviate the economic burdens that often prevent vulnerable families from seeking necessary medical attention.



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Key Performance Indicators

87.5%

Institutional Birth Rate

2019 national average

8.69%

Growth Since 2015

Four-year progress

7.48%

Target Gap

Next phase goal - 2023

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The Equity Gap

Aspirational vs. Standard Districts

Success Story

Aspirational districts performing same as standard districts

- 2015: Standard 80% vs Aspirational 69%
- 2019: Standard 89% vs Aspirational 81%

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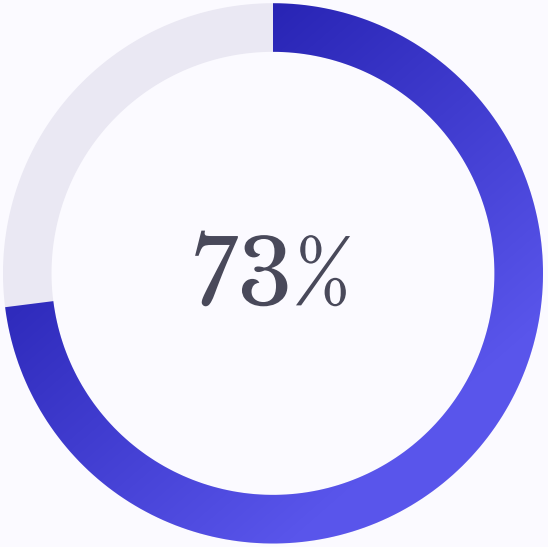
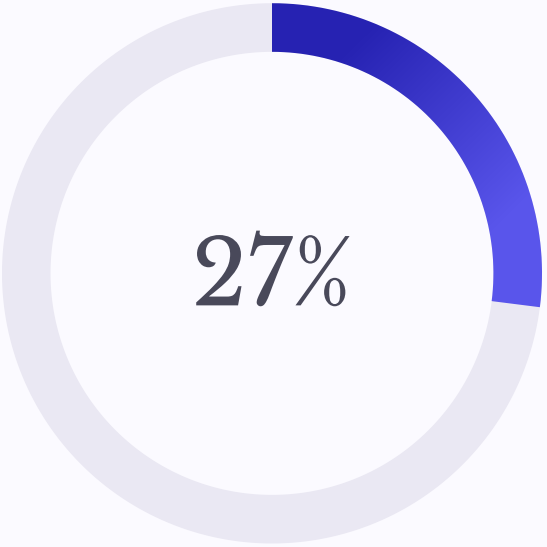
Public vs. Private Facilities

Where Most Deliveries Happen



Public Facilities

Government handles majority



Private Facilities

Supplemental care

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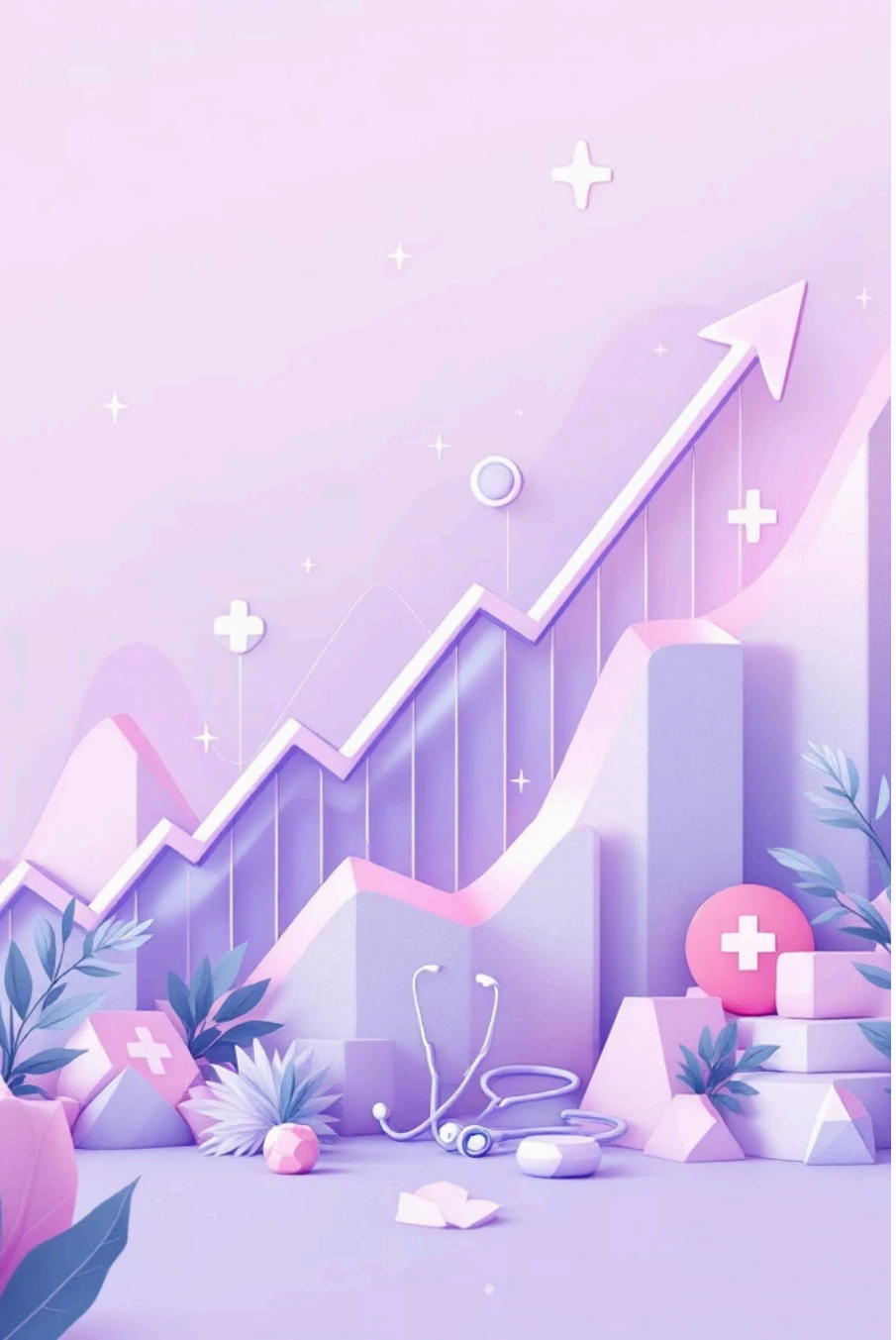

National Progress Trajectory

2015 to 2023 Forecast

1

2015: Baseline

78.84% institutional births

2

2019: Growth

87.52% across 700+ districts

3

2023: Target

96.21% projected 8.6% gain

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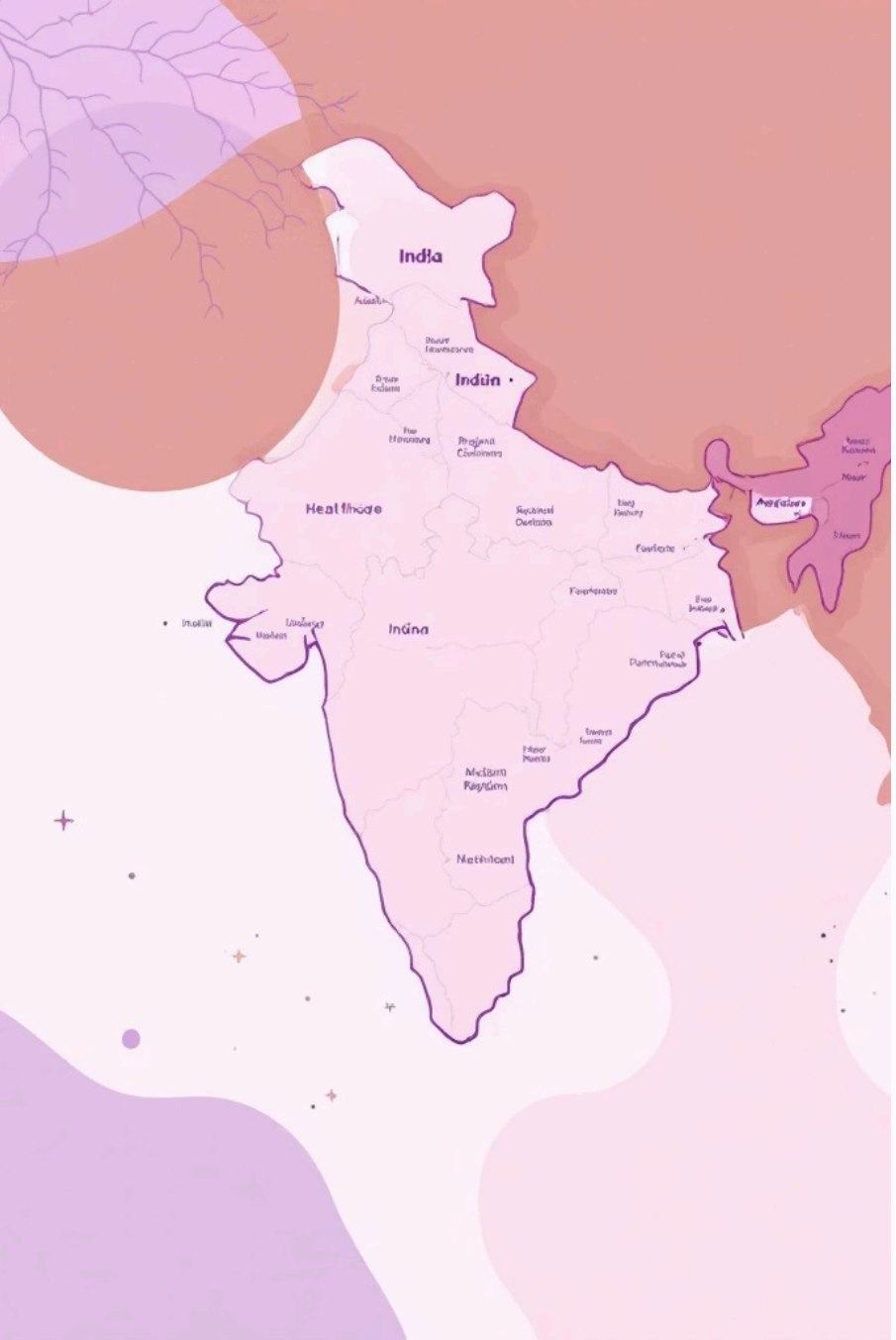

State-Level Audit

Leaders & Laggards

Top Performers

Kerala, Puducherry, Tamil Nadu: 97-100% institutional births

Priority States

Nagaland, Meghalaya, Manipur: 30-59% IB Rate - Need to find reasons for low Institutional Birth rates.

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The Financial Deterrent

High Cost of "Free" Healthcare

Inverse Correlation

Higher OOP costs = lower hospital deliveries

Reasons

Might be heavy spending on travel and hospital fees acting like a regressive tax

Strategic Gap

Financial protection must cover transport and incidental costs

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The Service Barrier

Registration ≠ Retention

01

MCP Card Registration

Entry point into healthcare system

02

ANC Drop-off

Significant decline in 4 visits

03

Lost Contact

Critical juncture failure

04

Continuity Model

Continuity of care needed

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Infrastructure Bottleneck

Physical Proximity = Primary Enabler

Key Finding

Functional PHCs drive ANC coverage decrease

Low facility density = decline in visits

This demonstrates that clinic access is the primary driver, while education and willpower remain secondary

Policy Priority: Functionalize Sub-Centers into Health and Wellness Centers

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Action Plan

Strengthen PHCs

Make existing Sub-Centers functional, not just new construction

Service Continuity

Use MCP cards to track and nudge mothers missing 3rd/4th ANC visit

Financial Protection

Expand cashless schemes to cover transport and incidental medicines

> "Data shows the path; policy must pave the road"

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